



An Outcomes-based Accreditation Framework Across 14 Countries: Lessons Learned

September 2026 Update

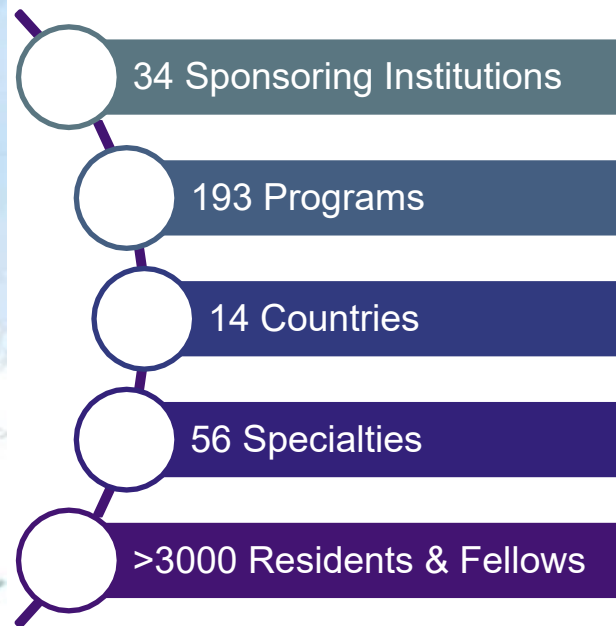
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Professor of Medicine Emeritus
Brown University, USA



Disclosures

Nothing to disclose.

The ACGME-I Community



ACGME International (ACGME-I)

Dual Functions of Assurance and Quality Improvement

The **Mission** of ACGME-I is to improve health care by assessing and advancing the quality of resident physicians' education through accreditation.

- Honesty and Integrity
- Accountability and Transparency
- Equity and Fairness
- Diversity and Inclusion
- Excellence and Innovation
- Stewardship and Service
- Leadership and Collaboration
- Engagement of Stakeholders
- ***Sensitivity to Local Culture and Medical Environment***



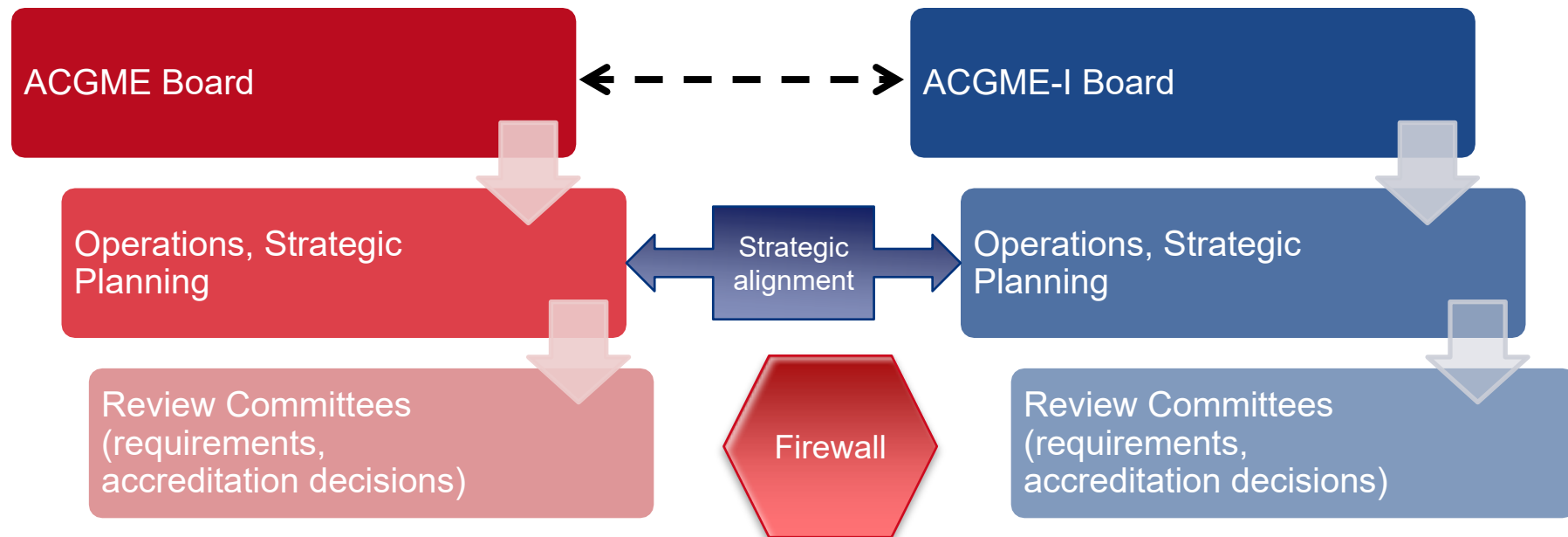


Lesson 1: It is important to be guided by mission and values

- Our primary goal is to serve the institution or country that engages with us; their goals vary widely.
- Our mission is not to recruit physicians to North America, but expanding opportunities for resident graduates, including mobility, is aligned with our values.
- Mission and values help us define our scope, the limits of flexibility, and our accreditation processes.
- We view the ACGME-I framework as one model, not a universal solution. We seek to support structured PGME while learning with and from our partners.



ACGME and ACGME-I: Separate but related





ACGME-I Framework

“Substantially similar” to ACGME framework

ACGME-International: Overall Framework



Structured Approach to PGME



ACGME framework adapted for use in global arena



Application of **single set** of global standards with sensitivity to local context

Five Important Features



Peer Review



Assurance



Quality Improvement



Milestones



Sponsoring Institute as Foundation

Requirements (Standards) Similar to ACGME



PROGRAM
(Specialty-Specific)

PROGRAM
(Foundational [Core])

INSTITUTIONAL

Alignment with WFME standards

Often coexistent with national standards

Ability to benchmark

Enhanced analytics and data support under development



Framework of ACGME-I Requirements (Standards)

Programs

Institution/Oversight

Resources / Personnel

Resident Appointment

Educational Program

Evaluation

Learning & Working Environment

Standards
for
Institution &

Programs
Same or Similar

framework to
ACGME, aligned
w/ WFME
principles

Same requirements
across all ACGME-I
institutions (*not
country or region
specific*)

Institutions

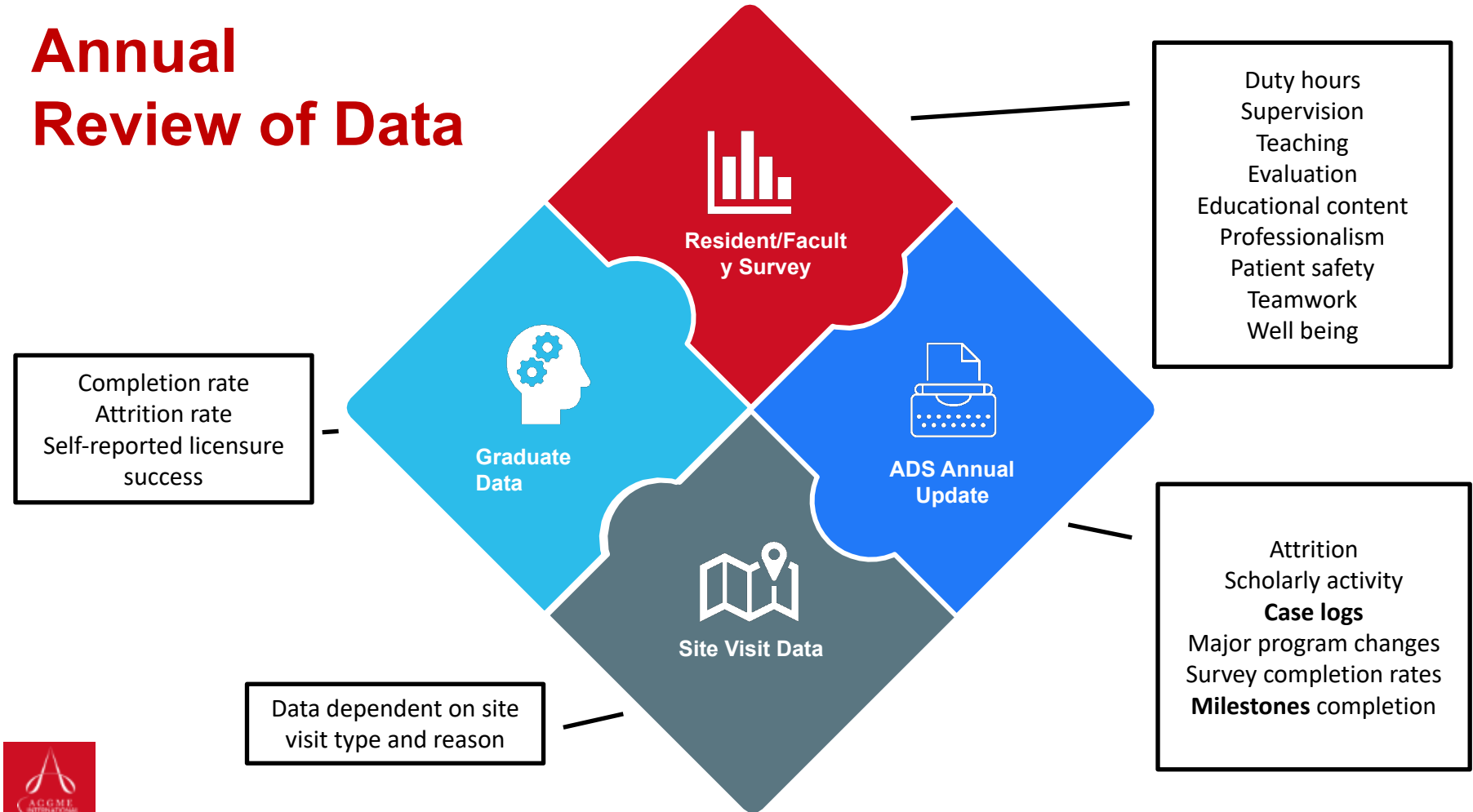
Organization & Responsibilities
of SI

Residents / Fellows

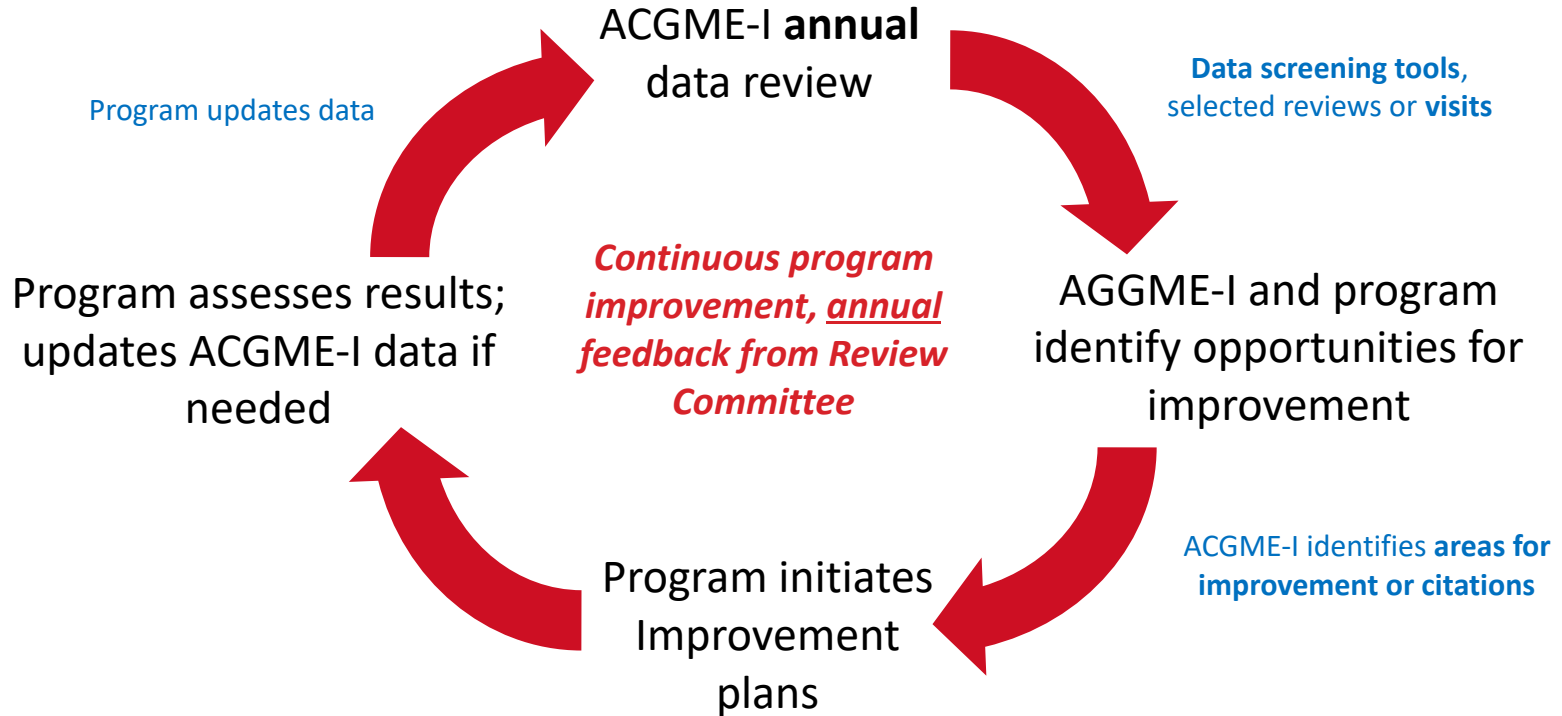
Clinical Learning & Working
Environment

GME Committee

Annual Review of Data

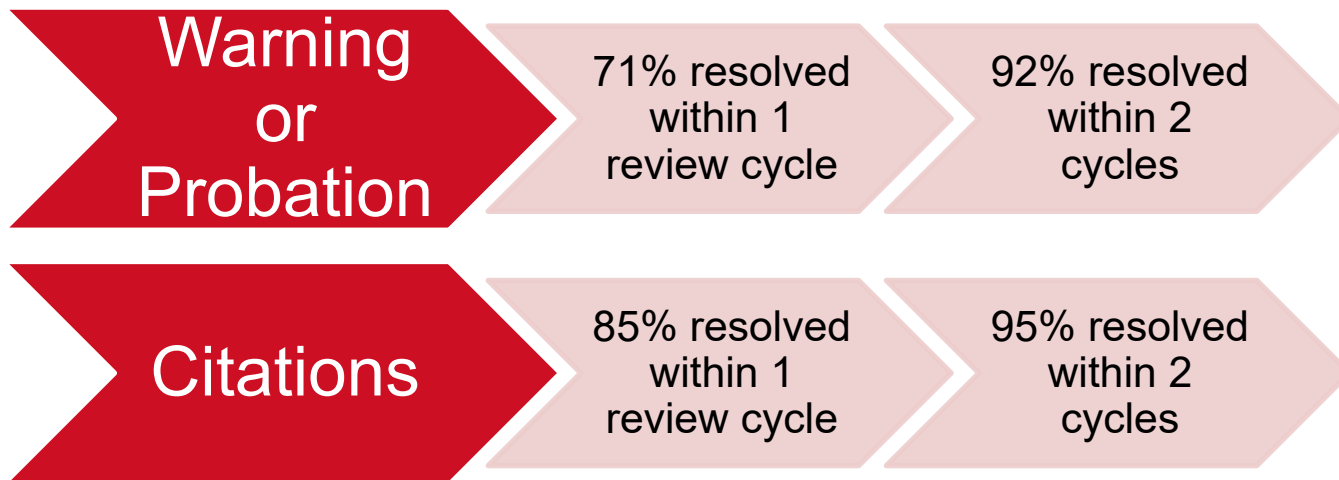


Continuous Quality Improvement Cycle of ACGME International





The Quality Improvement Aspect of ACGME-I Accreditation





Lesson 2: Our model is adaptable to the global context, but not without challenges

- Processes must be adapted to the global context, but the overall ACGME “approach” seems to work well when applied in a global context.
- It is sometimes difficult to balance flexibility (“glocalization”) with uniformity.
- Data-driven processes, QI, peer review, and transparency are critical elements.
- Our current “tiered” accreditation framework (foundational/core → specialty-specific) has rarely been used. This may mean that foundational accreditation itself is the major hurdle or that we need to rethink how the tiers are structured.
- It is important to recognize the potential differences in training across our institutions, particularly: cultural, medico-legal, systems-based practice, and scope of practice for some specialties.

ACGME-International: What is our role?



- Promotion of structured PGME
- Recognition of quality PGME
- Complement local or regional accreditors
- Facilitator of physician mobility or increasing opportunities for graduates 
- Building a PGME global community; facilitating cross-border educational partnerships, providing faculty development.
- Determining “substantial similarity” as coined by FSMB (and analogous process in Canada) (202)



Country X

Milestones Data for Benchmarkin

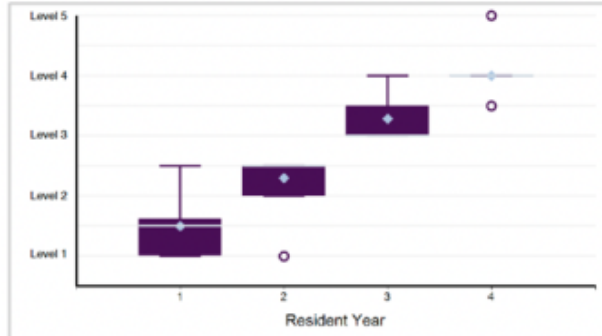
g

USA

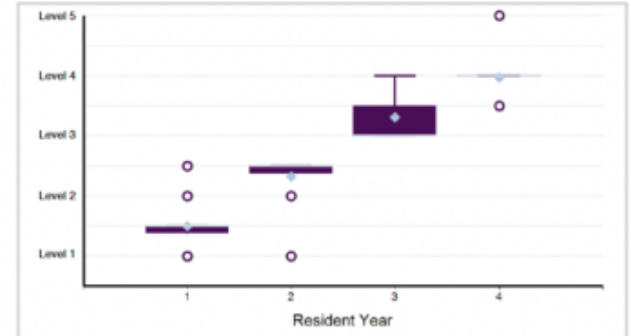
Specialty: **Emergency medicine**

Resident Year	1	2	3	4	Total Residents
# of Residents	28	20	21	16	85

1. Patient Care - 1 - Emergency Stabilization - Prioritizes critical initial stabilization action and mobilizes ...



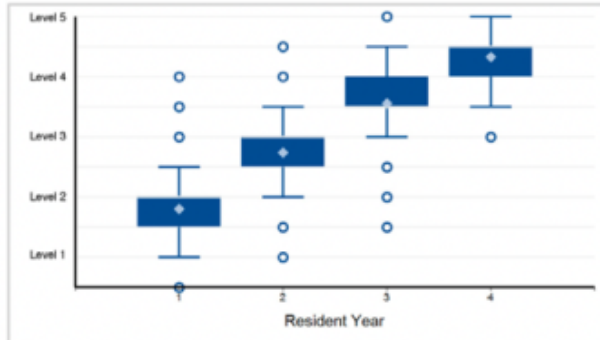
2. Patient Care - 2 - Performance of Focused History and Physical Exam - Abstracts current findings in ...



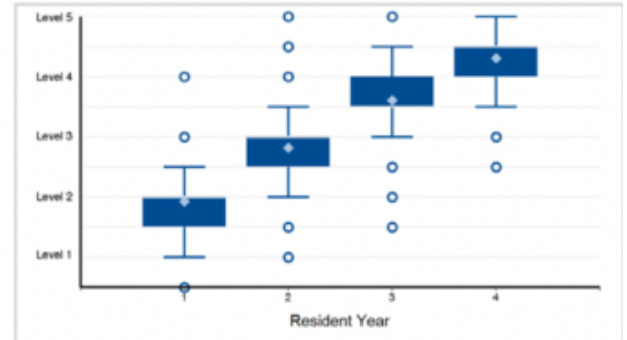
Resident Year	1	2	3	4	Total Residents*
# of Residents	672	659	645	633	2,609

*Note: There are 32 residents with an incomplete milestone evaluation that are excluded from the box plots below.

1. Patient Care - Patient Care 1: Emergency Stabilization



2. Patient Care - Patient Care 2: Performance of a Focused History and Physical Exam



RESEARCH

Open Access

Revamping the educational environment and engagement: How ACGME-I accreditation transforming training programs in Qatar

Shieen Suliman^{1,2,3,4*}, Anas Adel Kallah¹, Muthusamy Thiruvengadam Vijay Jegarathi¹, Abdulla Alsuad², Daniel Robert Mills², Kristen Jennifer Al-Ameri¹, Husameldin Mohamed Ali Rudwan³ and Abdulkarim Al-Khal⁵

Suliman et al. BMC Medical Education (2025) 25:58
https://doi.org/10.1186/s12909-025-06813-x

BMC Medical Education

RESEARCH

Open Access

International accreditation of a pediatric hematology/oncology fellowship program in Guatemala: a quantitative assessment

Daniel C. Moreira^{1*}, Claudia Garrido², Roy Rosado², Leeanna Fox Irwin³, Ricardo Mack^{2,3}, Carlos Rodríguez-Gallardo² and Federico Arzón-Ruizman^{1,3}

GME AROUND THE WORLD

Aligning Graduate Medical Education With the Health Care Needs of Abu Dhabi: One Decade After the Restructuring Initiative

Hatem Alamen¹ MD, FRCPC

International Journal of Medical Education. 2015;6(12):124
ISSN: 2042-6372
DOI: 10.5134/ijme.2015.11.06

Medical students' perceptions of international accreditation

Halal Ibrahim¹, Sawan Abdel-Razig², Satish C Nair³

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²Langone School of Medicine, New York University, USA



Viewpoint

ACGME-I Accreditation in Low-Resource Training Programs

Sayed K. Ail, MD¹, Ali K. Chandani, MD¹, Ishak Mansi, MD²

Rise et al. BMC Medical Education (2025) 25:912
https://doi.org/10.1186/s12909-025-07990-2

BMC Medical Education

RESEARCH

Open Access

Competency-based postgraduate medical education as an institutional initiative in a low- middle-income country: program directors' perceptions of early implementation

Qamir Riaz^{1*}, Muhammad Rizwan Khan² and Mushtaq Ahmed³

ACGME NEWS AND VIEWS

The Positive Effects of Accreditation on Graduate Medical Education Programs in Singapore

Kathleen D. Holt, PhD
Rebecca S. Miller, MS
Lauren M. Byrne, MPH
Susan H. Day, MD



See our website:
www.acgme-i.org

About Us

What is Accreditation

Accreditation Process

Institutions

Specialties

ACGME International Home > What is Accreditation > Scholarly Articles on the Impact of ACGME-I Accreditation

Scholarly Articles on the Impact of ACGME-I Accreditation

Evidence of Positive Impact of ACGME-I

Educational outcomes

Advancing CBME

Addressing workforce needs

Improving medical student & clinical educator experience

Development of PGME in low resource areas



Lesson 3: Growth and increasing diversity of clients necessitates frequent strategic planning updates

- We have demonstrated proof-of-concept that a single set of standards (not country-specific) can be applied to a range of cultures and settings.
- Our “role” is determined largely by the institutions that engage with us (“value added”), and/or external factors that we do not control (e.g., ITP pathways in US).
- At present, we have not determined our role, in supporting programs in the development phase or those with a substantially different PGME framework.
- Building a global GME community is a strong ancillary benefit to most of our partners.
- **Programs seeking accreditation from us wish for a certification option to recognize graduate achievement.**



Fellowship Data

ACGME-I residency graduates who complete fellowship in US



ACGME-I Fellowship Pathways

151

**first-year U.S.
fellowship
matriculations**

125

**unique ACGME-I
residency
graduates**

Some graduates completed more than one fellowship during the study period.

32 receiving institutions had more than one matriculation

80

**receiving
institutions**

37

U.S. states

46

**fellowship
subspecialties**

14

**core residency
specialties**

The pathway is being used across multiple specialties, institutions, and regions.





14 residency specialties → 46 fellowship subspecialties

Residency specialties represented

14

Anesthesiology

Dermatology

Diagnostic Radiology

Emergency Medicine

Family Medicine

Internal Medicine

Neurology

Orthopaedic Surgery

Otolaryngology

Pathology

Pediatrics

Psychiatry

Surgery

Urology

Fellowship subspecialties entered

46

ANESTHESIOLOGY

Adult cardiothoracic anesthesiology • Critical care medicine (Anesthesiology) • Obstetric anesthesiology • Pediatric anesthesiology • Regional anesthesiology and acute pain medicine

EMERGENCY MEDICINE

Emergency medical services • Pediatric emergency medicine

INTERNAL MEDICINE

Critical care medicine (Internal medicine) • Endocrinology, diabetes, and metabolism • Geriatric medicine • Infectious disease • Nephrology • Transplant hepatology

MULTIDISCIPLINARY

Hospice and palliative medicine • Sleep medicine (multidisciplinary)

NEUROLOGY

Clinical neurophysiology • Epilepsy • Neuromuscular medicine

ORTHOPAEDIC SURGERY

Adult reconstructive orthopedic surgery • Foot and ankle orthopedics • Hand surgery • Orthopedic sports medicine • Orthopedic surgery of the spine • Pediatric orthopedic surgery

OTOLARYNGOLOGY

Pediatric otolaryngology

PSYCHIATRY

Child and adolescent psychiatry • Consultation-liaison psychiatry • Geriatric psychiatry

RADIOLOGY

Abdominal radiology • Neuroradiology • Nuclear medicine • Pediatric radiology

PEDIATRICS

Developmental-behavioral pediatrics • Neonatal-perinatal medicine • Pediatric critical care medicine • Pediatric endocrinology • Pediatric gastroenterology • Pediatric hematology/oncology • Pediatric infectious diseases • Pediatric nephrology • Pediatric pulmonology

DERMATOLOGY

Pediatric dermatology

PATHOLOGY

Molecular genetic pathology (multidisciplinary)

SURGERY

Surgical critical care • Surgical oncology

OB/GYN OR UROLOGY

Urogynecology and reconstructive pelvic surgery





Summary

- The accreditation process of ACGME-I has been applied globally across a range of institutions and appears to be valuable in both the assurance and quality improvement functions in selected GME programs.
- The value that ACGME-I brings to its SIs is variable, defined largely by the SI's goals for GME, and challenges us to constantly think of the "value proposition" of accreditation.
- There is an increasing focus on PGME in many areas of the world; there is an opportunity for accreditors, certification boards, and medical educators to build community, learn from each other, and improve GME training.

Backup Slides



Specific Program Requirements

ACGME → ACGME-I

